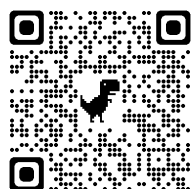




A FIRST IN HUMAN STUDY WITH THE NOVEL MET KINASE INHIBITOR, DO-2

IN ADULT PATIENTS WITH ADVANCED OR REFRACTORY MET
PATHWAY ACTIVATED SOLID TUMORS

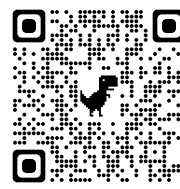
[NCT05752552](https://www.clinicaltrials.gov/ct2/show/study/NCT05752552)



PROTOCOL DO-2.22.01

STUDY LEAFLET

[Heyleys.nl](https://www.heyleys.nl)



PLEASE CONSIDER REFERRING
YOUR MET-BIOMARKER POSITIVE PATIENTS TO

Principal Investigator in The Netherlands



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Currently approved MET inhibitors for patients with NSCLC harboring MET exon 14 skipping mutation are limited by **poor tolerability** leading to frequent dose reductions/interruptions/halting.

The toxicity is mainly due to the high levels of circulating drug (active and inactive metabolites) following the high daily dose.

DO-2 is a deuterated MET kinase inhibitor

Replacing an essential Hydrogen atom by a Deuterium atom which

- significantly reduces metabolic liability
- extends the efficacious plasma exposure profile

The starting dose (5mg) is predicted to be efficacious.

DO-2 is a highly efficacious MET kinase inhibitor in pre-clinical models



DO2-2.22.01 Study Objective

To generate clinical proof of concept that blocking metabolic liabilities improves clinical tolerability and activity.

Study Considerations



- 2-part study
- Dose escalation
- 15 evaluable patients
- Simon Accelerated Titration Design 3



- Starting dose: 5mg
- Orally once daily for 28 days
- 4 weekly cycles

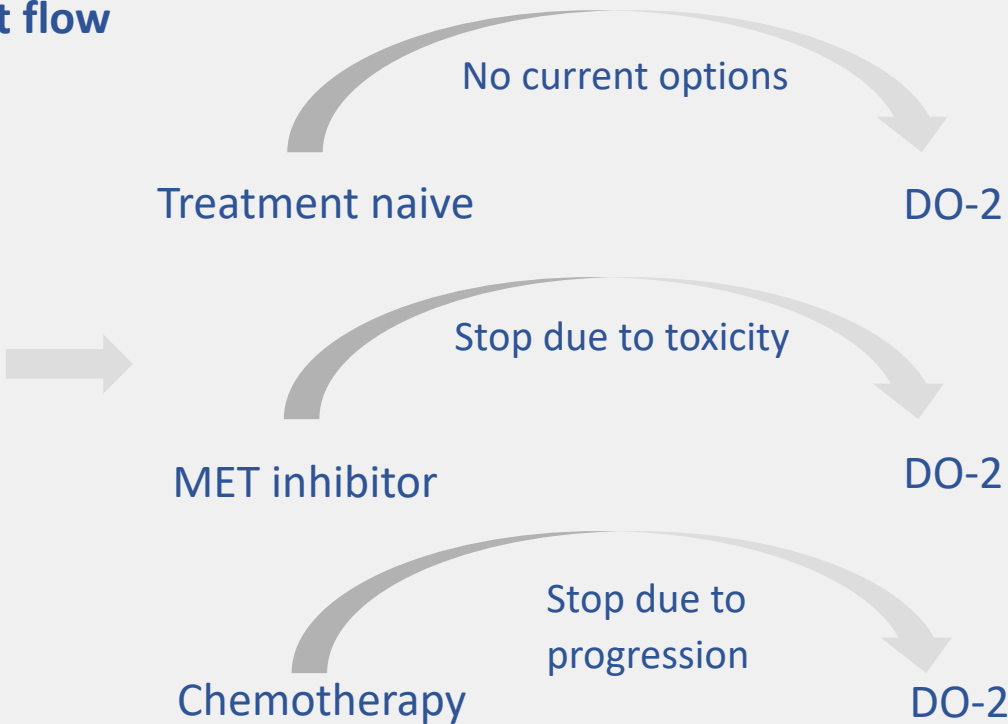


- Patients travel costs will be fully reimbursed

Examples of Patient flow



Known MET alteration status from archived tissue

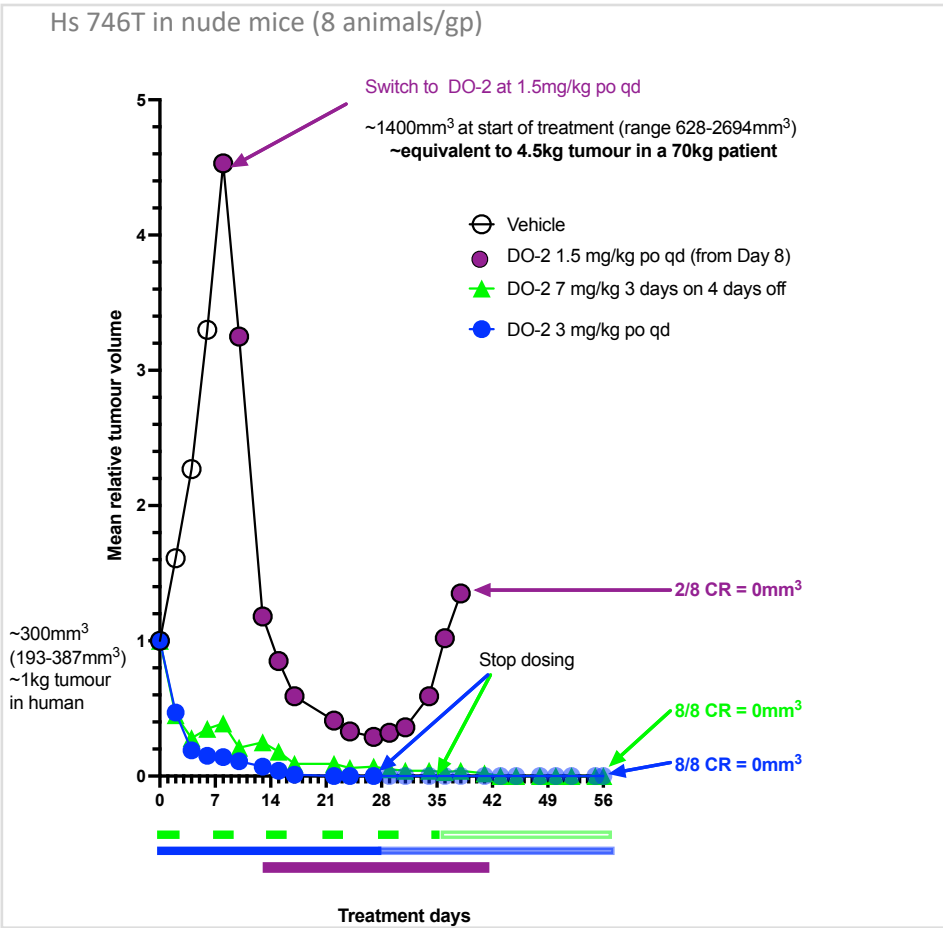


DO-2: highly efficacious MET kinase inhibitor with complete responses in pre-clinical models

Uninterrupted daily dosing results in best response

DO-2 drives complete responses in bulky tumours

MET Exon 14 skipping mutant human xenograft model in immuno-compromised mice

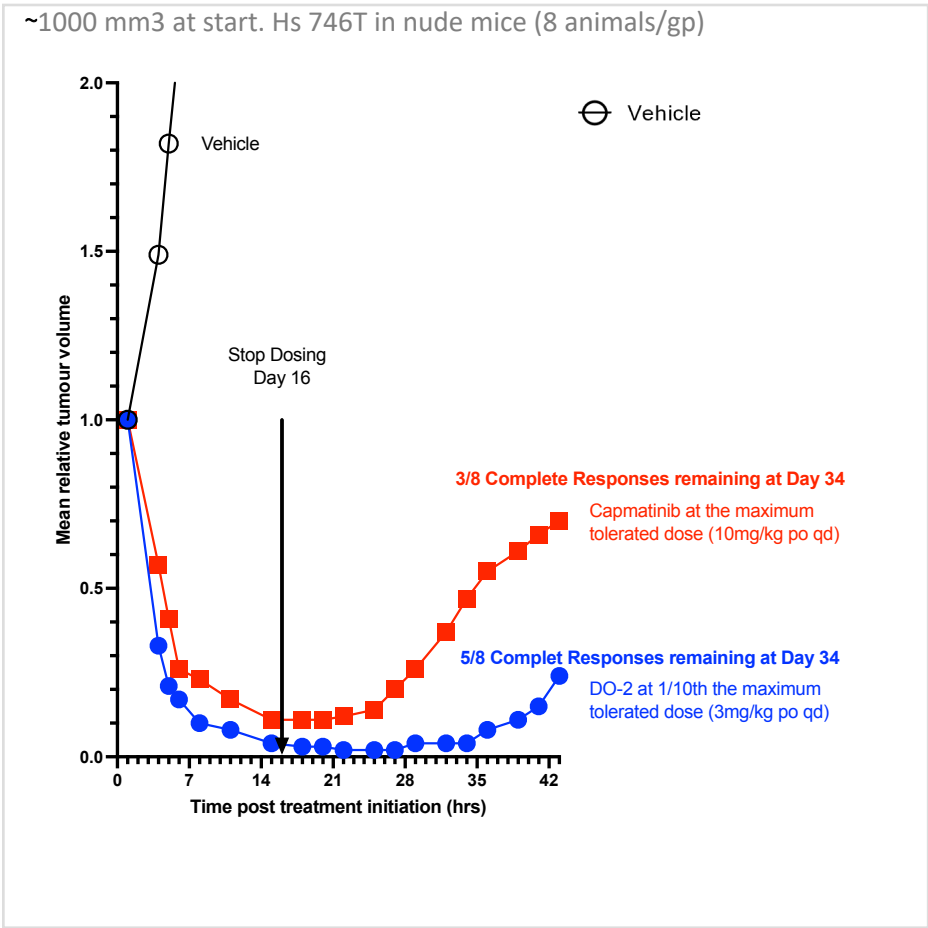


Animals bearing very bulky tumours (corresponding to ~4.5kg in a patient) respond at a human equivalent daily oral dose of ~5mg.

Animals bearing large tumours (corresponding to ~1kg in a patient) are completely eradicated at a human equivalent daily oral dose of ~15mg.

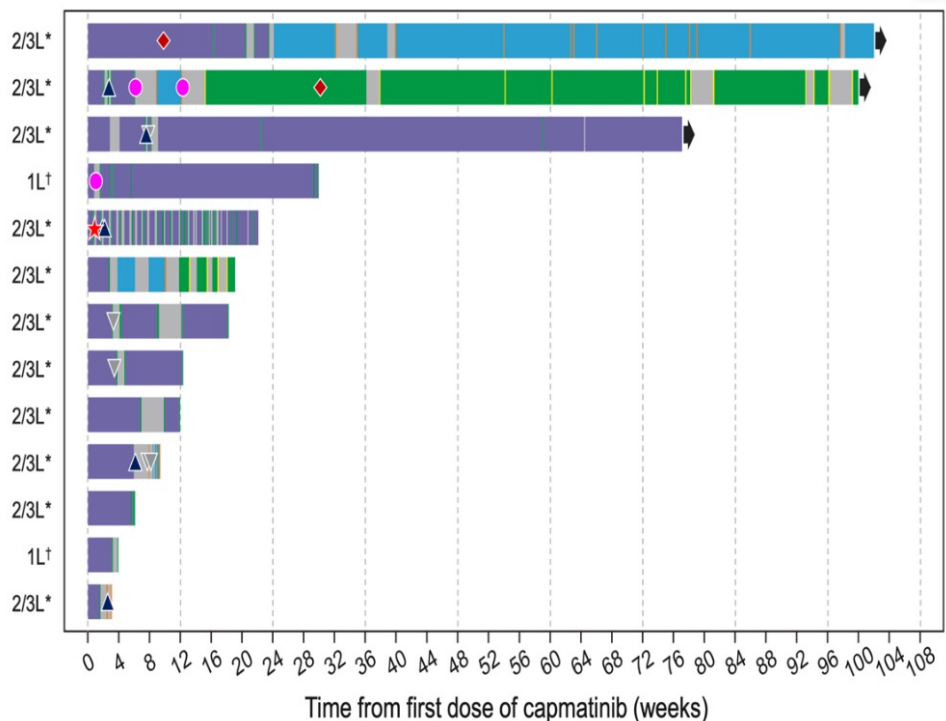
DO-2 is more potent and efficacious than capmatinib

MET Exon 14 skipping mutant human xenograft model in immuno-compromised mice



DO-2 is more efficacious when dosed at <10% of the maximum tolerated dose compared to capmatinib dosed at it's maximum toleraterd dose in mice.

Main competitor poorly tolerated; frequent dose reductions and interruptionsl



Swimmer plot showing duration of capmatinib administration and timing of common adverse events (AEs) according to treatment line in 13 patients with METΔex14-mutated non-small-cell lung cancer. Data cutoff: September 18, 2019.

Seto et al, Cancer Sci. 2021 Apr;112(4):1556-1566.
doi: 10.1111/cas.14826.

*Received capmatinib as second-/ third-line therapy (cohort 4).
† Received capmatinib as first-line therapy (cohort 5b)

Main Criteria for enrolment

- Confirmed MET activating mutations (NGS, WES, WTS or other genomic analysis methods),
or
- Confirmed MET amplification (≥ 10 copies) on archived tumor tissue
or
- Hereditary Renal Papillary Cancer

- ECOG ≤ 2 (Performance score: ambulatory and capable of all self-care but unable to carry out any work activities, up and about more than 50% of waking hours)

- Normal organ functions

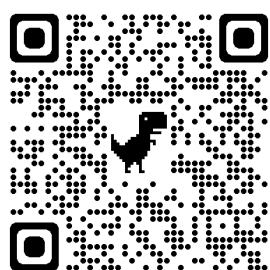
- Patients with brain metastases can be included provided all of the following criteria are met:
 - CNS lesions are asymptomatic and previously treated
 - No ongoing requirement for corticosteroids as therapy for CNS metastases
 - Imaging demonstrates stability of disease > 28 days from last treatment for CNS metastases
 - Leptomeningeal involvement (leptomeningeal carcinomatosis)

THANK YOU FOR DISCUSSING THIS STUDY WITH YOUR ELIGIBLE PATIENTS

FOR MORE DETAILS

CLINICALTRIALS.GOV

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